LoanBazaar®

Secure Distribution Venture Pvt. Ltd.

What Is Hospitality & Healthcare Funding ?

Developed by www.loanbazaaronline.com

Table of Contents & Master Outline

Module	Main Heading	Page No
Module 1	Product Overview & Core Fundamentals of Hospitality & Healthcare Funding	3
Module 2	Structural Architecture & Sub-Sector Financing Mechanics	5
Module 3	Regulatory, Statutory, Tax & Insurance Framework in India	8
Module 4	Common Eligibility Criteria & Institutional Underwriting Parameters	10
Module 5	Security Architecture, Collateral Controls & Documentation Suite	12
Module 6	Tenures, Pricing, Suitable Sectors, Advantages & Risks	14
Module 7	Tenures, Pricing, Suitable Sectors, Advantages & Risks	14
Module 8	Tenures, Pricing, Suitable Sectors, Advantages & Risks	14
Module 9	Comprehensive Real-World Case Study (INR 300 Crore Blended Facility)	16
Module 10	Legal Enforcement, Default Protocols & IBC 2016 Integration	18
Module 11	Rookie Mastery Examination (15 High-Yield Test Q&As)	19

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 1: Product Overview & Core Fundamentals of Hospitality & Healthcare Funding

1.1 Executive Definition & Strategic Significance

Core Definition: Hospitality & Healthcare Funding represents a specialized category of wholesale corporate credit, infrastructure term debt, structured private credit, and equipment lease financing engineered specifically to meet the high-capital, long-gestation, and specialized asset requirements of India's hospitality and healthcare sectors. These two sectors form the backbone of India's social infrastructure and service economy, encompassing multi-specialty and single-specialty hospital chains, diagnostic networks, luxury and business hotels, boutique eco-resorts, and senior living facilities.

Sectoral Distinctiveness: Unlike standard corporate term debt or general commercial real estate financing, funding for hospitality and healthcare projects requires specialized underwriting capable of addressing long construction timelines, high upfront capital intensity, specialized medical and building accreditations, key human capital dependence (senior surgeons and hospitality brand operators), and distinct cash flow dynamics such as seasonal occupancy variations in hotels or third-party administrator (TPA) and insurance payment cycles in hospitals.

1.2 Infrastructure Status & The Regulatory Paradigm Shift

Infrastructure Classification: A foundational pillar of Indian hospitality and healthcare financing is the government's grant of Infrastructure Status to specific sub-segments under the Harmonized Master List of Infrastructure Sub-Sectors. Specifically, hospitals with a minimum capacity of 100 beds (or lower thresholds in Tier-2/3 cities) and hotels rated 3-star and above enjoy infrastructure classification. This statutory designation unlocks longer loan tenures (up to 15–20 years), lower risk-weighted capital requirements for commercial banks, access to External Commercial Borrowings (ECB), and eligibility for long-term pension fund and insurance debt capital.

The Institutional Credit Gap: Despite infrastructure status, traditional commercial banks face strict single-borrower and sectoral exposure caps imposed by the Reserve Bank of India (RBI). Consequently, specialized Non-Banking Financial Companies (NBFCs), SEBI-registered Category II Private Credit Alternative Investment Funds (AIFs), and Real Estate/Infrastructure Debt Funds have emerged as dominant institutional providers of customized, structured debt facilities for brownfield expansions, brand flagging, medical equipment acquisitions, and promoter buyout transactions.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

1.3 Comparative Product Taxonomy Matrix

To establish clear analytical boundaries for financial professionals, the table below compares Hospitality & Healthcare Funding against traditional commercial term loans, real estate project finance, and general equipment lease financing.

Dimension	Hospitality & Healthcare Debt	Commercial Term Loans	Real Estate Project Debt	Equipment Lease Financing
Primary Underwriting Basis	Operating EBITDA, ARPOB/RevPAR, occupancy & asset cover	Historical 3-year net profit & physical collateral	Residential/commercial unit pre-sales & construction speed	Standalone liquidation value of machinery
Typical Loan Tenure	7 to 15 Years (Long-term infrastructure tenure)	3 to 5 Years (Medium-term amortizing loan)	3 to 5 Years (Short-term construction bridge)	3 to 7 Years (Matching equipment useful life)
Principal Moratorium	24 to 36 Months (Covers construction & ramp-up)	3 to 6 Months (Minimal grace period)	12 to 24 Months (Covers construction period)	Zero to 3 Months (Immediate lease payments)
Collateral Architecture	Specialized real estate, FF&E, medical gear & TRA escrow	General fixed assets & corporate guarantees	Pledge of project land, unsold inventory & escrow	Hypothecation over specific leased equipment
Tax Incentives	Section 35AD (100% capex deduction) & Sec 36(1)(iii)	Section 36(1)(iii) standard interest deduction	Limited tax incentives (Standard interest deduction)	Depreciation & lease rental tax deduction
Primary Cash Flow Source	IPD/OPD cash, TPA claims, room & F&B receipts	General operating cash flows of corporate	Customer milestone escrows & flat booking sales	General corporate cash flows

Analytical Takeaway: The matrix highlights that Hospitality & Healthcare Funding bridges the gap between pure real estate project debt and operational corporate credit, offering extended tenures and structured moratoriums suited to long asset gestation.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 2: Structural Architecture & Sub-Sector Financing Mechanics

2.1 Structural Mechanics for the Hospitality Sub-Sector

Hospitality Overview: Hospitality projects are characterized by high initial development costs and operational cash flows tied directly to location, Average Daily Rates (ADR), Revenue Per Available Room (RevPAR), and Food & Beverage (F&B) banquet yields. Debt structures are customized across three main operational phases:

1. Greenfield Development Debt: Financing land acquisition, civil construction, and interior fit-outs. Requires 24–36 month principal moratoriums matching the construction timeline. Secured by first legal mortgages over land, building hypothecation, and pledge of sponsor shares.

2. Brownfield Expansion & Rebranding Debt: Funding hotel upgrades, room expansion, and brand conversion/flagging to join international chains (e.g., Marriott, Taj, IHG, Hyatt, Accor). These facilities feature 12–18 month moratoriums and are underwritten on projected post-rebranding ADR uplifts.

3. LRD & Operating Cash Flow Monetisation: Once a hotel achieves stable occupancy (>65%), operating cash flows are securitized via Lease Rental Discounting (LRD) or structured cash flow monetisation. Room and event credit card receipts flow directly into a lockbox Escrow Trust Account.

2.2 Structural Mechanics for the Healthcare Sub-Sector

Healthcare Overview: Healthcare assets (hospitals, diagnostic chains, single-specialty centers) display defensive cash flow characteristics, low economic cycle correlation, and steady volume growth driven by rising health insurance penetration and medical tourism. Debt structures focus on three distinct areas:

1. Hospital Infrastructure Term Debt: Funding civil construction for multi-specialty hospitals, cleanroom operating theaters, and intensive care units (ICU). Underwritten on bed capacity, target occupancy, and regional doctor availability.

2. Specialized Medical Equipment Debt: Financing advanced medical diagnostic and surgical technology (e.g., Linear Accelerators for Oncology, PET-CT, 3T MRI, Cath Labs, Robotic Surgical Systems). Structures often utilize equipment hypothecation with 5-to-8 year repayment matching equipment obsolescence.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

3. TPA & Insurance Receivables Discounting: Hospitals often face working capital drag due to 45-to-90 day payment cycles from Third Party Administrators (TPAs), private health insurers, and government schemes (CGHS, ECHS, Ayushman Bharat / PM-JAY). Discounting facilities provide instant liquidity against verified insurance claims.

2.3 Priority Escrow Trust & Retention Account (TRA) Cash Waterfall

TRA Mechanism: To isolate credit risk and ensure uninterrupted debt servicing, lenders mandate a legally binding Trust & Retention Account (TRA) Escrow Agreement among the borrower, lender, and an escrow agent bank. All operating revenues—room collections, credit card merchant swipes, IPD/OPD patient billings, TPA remittances, and banquet bookings—must be deposited directly into the TRA Nodal Account.

The table below outlines the rigid statutory and contractual cash waterfall enforced on a monthly basis before any surplus cash can be released to promoters.

Priority	Waterfall Level	Functional Description & Ring-Fencing Rules
Level 1	Statutory Taxes & Levies	Direct transfer of GST, Municipal Property Taxes, and employee TDS into statutory tax sub-accounts.
Level 2	Approved Essential Operating Expenses	Release of pre-approved monthly O&M budget (hospital medicine/consumables, utility bills, hotel food supplies, staff salaries).
Level 3	Senior Debt Service (Interest & Principal)	Mandatory transfer to Debt Service Sub-Account to service monthly interest and principal amortization of senior loans.
Level 4	Subordinated / Mezzanine Debt Service	Payment of cash coupons and scheduled principal on structured private credit or mezzanine debentures.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Priority	Waterfall Level	Functional Description & Ring-Fencing Rules
Level 5	Debt Service Reserve Account (DSRA)	Replenishment of DSRA sub-account to maintain 3 to 6 months of future debt service in liquid fixed deposits.
Level 6	Equipment Replacement & Maintenance Reserve	Transfer of 2%–4% of gross revenues into a dedicated CapEx reserve for medical gear maintenance or hotel FF&E updates.
Level 7	Cash Sweep Acceleration (if triggered)	If DSCR drops below 1.25x or occupancy falls below 50%, 50%–100% of remaining cash is swept to prepay principal.
Level 8	Surplus Release to Promoters	Unconditional release of residual cash to corporate borrower accounts for dividends or expansion, provided zero default exists.

Risk Mitigation: The TRA cash waterfall ensures that operating essential expenses and debt servicing take precedence over promoter distributions, protecting lender capital against cash diversion.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 3: Regulatory, Statutory, Tax & Insurance Framework in India

3.1 Reserve Bank of India (RBI) Regulations & Infrastructure Rules

RBI Regulatory Baseline: The Reserve Bank of India regulates commercial bank and NBFC exposures to hospitality and healthcare through specific credit facility guidelines. Key regulatory parameters include:

Infrastructure Credit Rules: Hospitals with 100+ beds (or 50+ beds in Tier-2/3 cities) and hotels 3-star and above qualify under the RBI Harmonized Infrastructure List. This allows lenders to assign lower Risk-Weighted Assets (RWA) and extend loan tenures up to 15 years.

Exposure Caps & Consortiums: Commercial banks must comply with RBI Large Exposure Framework (LEF) caps, limiting single-borrower exposure to 20% (expandable to 25% for infrastructure) of bank Tier-1 capital. This encourages multi-bank consortiums or private credit co-investments for large medical/hotel developments.

3.2 Healthcare Statutory Compliance & Accreditation

Healthcare Licensing: Healthcare facilities must comply with rigorous statutory licensing regimes. Underwriters treat non-compliance as an immediate Event of Default. Key statutory mandates include:

1. NABH Accreditation: National Accreditation Board for Hospitals & Healthcare Providers (NABH) accreditation is the gold standard required by health insurers, TPAs, and international medical tourists.

2. AERB Clearances: Atomic Energy Regulatory Board (AERB) mandatory radiation safety approvals for operating Radiology, CT, PET-CT, and Radiation Oncology equipment.

3. Environmental & Clinical Approvals: Mandatory registration under state Clinical Establishments Acts, compliance with Bio-Medical Waste Management Rules 2016, PC-PNDT Act compliance for ultrasound equipment, and Fire Safety NOCs.

3.3 Hospitality Statutory Compliance & Licensing

Hospitality Licensing: Hospitality properties operate under complex municipal and central regulatory frameworks. Key statutory prerequisites include:

Mandatory Approvals: FSSAI Food Safety Licenses for all restaurants and banquet kitchens; Excise & Bar Licenses for serving alcohol; Local Municipal Commercial Operation & Building

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Occupation Certificates (OC); Fire Safety NOCs; and Coastal Regulation Zone (CRZ) environmental clearances for beach resorts.

3.4 Income Tax Act Incentives & Statutory Frameworks

Section 35AD Capital Incentives: Under Section 35AD of the Income Tax Act, 1961, businesses engaged in building and operating a new hospital with at least 100 beds or a new 2-star or higher category hotel are eligible for a 100% tax deduction on all capital expenditure incurred (excluding land, goodwill, and financial instruments) during the financial year. This statutory incentive significantly boosts early-year cash flows and debt coverage.

Tax Shield & Corporate Compliance: Additionally, interest paid on term debt and debentures is 100% tax-deductible as finance costs under Section 36(1)(iii). Private credit facilities structured via Category II AIFs benefit from full tax pass-through status under Section 115UB. Issuance of debentures requires compliance with Sections 42, 71, and 77 (ROC Form CHG-1 filing within 30 days) of the Companies Act, 2013.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 4: Common Eligibility Criteria & Institutional Underwriting Parameters

4.1 Quantitative Underwriting Benchmarks

Institutional credit committees apply rigorous operational and financial benchmarks tailored to hospitality and healthcare assets. The table below details these quantitative parameters.

Underwriting Metric	Healthcare Sub-Sector Benchmark	Hospitality Sub-Sector Benchmark
Asset Capacity	Minimum 50 to 100+ operational beds	Minimum 50 to 200+ operational keys
Occupancy Rate %	Minimum 60% to 75%+ sustainable occupancy	Minimum 62% to 78%+ sustainable occupancy
Primary Yield Metric	ARPOB: INR 25,000 to INR 65,000+/day	ADR: INR 5,000 to INR 25,000+/night
Secondary Revenue Metric	ALOS: 3.5 to 5.0 days (Optimal bed turnover)	RevPAR: INR 3,200 to INR 18,000+/night
Auxiliary Revenue Share	OPD & Diagnostics: 20%–35% of total revenue	Food & Beverage (F&B): 25%–40% of total revenue
Operating EBITDA Margin %	18.0% to 28.0%+ sustainable EBITDA margin	25.0% to 38.0%+ sustainable EBITDA margin
Debt Service Coverage (DSCR)	Projected DSCR >= 1.35x to 1.60x	Projected DSCR >= 1.35x to 1.60x
Maximum Debt / EBITDA	Capped at maximum 3.50x to 4.50x Net Debt/EBITDA	Capped at maximum 3.50x to 4.50x Net Debt/EBITDA
Asset Cover Ratio (ACR)	Minimum 2.00x to 2.50x Tangible Asset Cover	Minimum 2.00x to 2.50x Tangible Asset Cover

Benchmark Significance: These quantitative thresholds ensure that the asset generates sufficient operational buffer to withstand occupancy fluctuations or seasonal drops without threatening debt service.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

4.2 Qualitative & Operational Governance Filters

Qualitative Governance: In addition to financial ratios, underwriters evaluate key qualitative operational risks:

1. Human Capital & Operator Agreements: For hospitals, evaluating doctor retention agreements, department head reputations (Oncology, Cardiology, Neurology), and clinical error insurance. For hotels, evaluating Hotel Management Agreements (HMAs) with international brand operators, franchise fee terms, and booking engine integration.

2. TEV Studies & Catchment Audits: Independent Techno-Economic Viability (TEV) study validating project cost estimates, catchment area population demographics, competitor bed/room density, and CIBIL Commercial rank (CMR 1 to CMR 4).

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 5: Security Architecture, Collateral Controls & Documentation Suite

5.1 The 5-Pillar Security Package

Security Overview: Hospitality and healthcare debt facilities feature a comprehensive multi-layered security package designed to provide 100% loss protection:

1. First Legal Real Estate Mortgage: First registered legal mortgage on commercial land, factory/hospital buildings, and hotel structures, providing minimum 2.0x asset cover.

2. Equipment & Movable Hypothecation: Hypothecation charge over high-value medical diagnostic equipment (MRI, CT, PET-CT, Cath Labs) or hotel Furniture, Fixtures & Equipment (FF&E).

3. Receivables & Escrow Charge: Exclusive charge over the Escrow TRA receiving account, TPA/insurance receivables, credit card merchant settlement accounts, and room booking engine cash flows.

4. Demat Share Pledge & NDU: Pledge of 30% to 51% dematerialized promoter equity shares held in NSDL/CDSL depositories with Non-Disposal Undertakings (NDUs).

5. DSRA & Promoter Guarantees: Creation of a Debt Service Reserve Account (DSRA) in liquid fixed deposits covering 3 to 6 months of principal and interest payouts, alongside joint and several personal/corporate promoter guarantees.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

5.2 Master Documentation Suite

The table below outlines the core legal contracts governing institutional hospitality and healthcare debt facilities.

Document Name	Statutory Framework	Core Purpose & Legal Provisions
Facility Agreement / DTD	Companies Act 2013 & Contract Act 1872	Sets credit terms, interest coupons, principal amortization, financial covenants, and default events.
Trust & Retention Agreement	Banking Regulations & Contract Act 1872	Establishes TRA Nodal Account, lockbox mechanics, and priority cash flow waterfall.
Hotel Management Agreement	Contract Act 1872 / Commercial Law	Tripartite agreement linking borrower, lender, and hotel brand operator (Marriott, Taj, etc.) ensuring brand continuity.
Deed of Hypothecation	Companies Act 2013 Section 77	Creates first charge on medical machinery, hotel FF&E, trade receivables, and inventories.
Deed of Mortgage	Transfer of Property Act 1882	Creates registered legal mortgage over commercial land, hospital buildings, and resort real estate.

Statutory Perfection: All security charges must be registered on ROC Form CHG-1 within 30 days under Section 77 of the Companies Act 2013, uploaded to CERSAI, and filed on the NeSL Information Utility portal.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 6, 7 & 8: Tenures, Pricing, Suitable Sectors, Advantages & Risks

6.1 Indicative Tenure Profiles & Principal Moratoriums

Tenure Calibration: Loan tenures are calibrated to the underlying asset lifecycle:

1. Greenfield Project Infrastructure Debt: 7 to 15 years tenure with 24–36 month principal moratoriums during civil construction and initial operational ramp-up.

2. Brownfield Expansion & Equipment Debt: 5 to 10 years tenure with 12–18 month moratoriums tailored to room expansion or new hospital wing additions.

3. Receivables & Working Capital Debt: 12 to 36 months revolving liquidity line against TPA insurance claims or hotel seasonal room bookings.

6.2 All-In Yield & Pricing Architecture

Pricing Spectrum: Pricing reflects lender type and underlying asset risk profile:

Commercial Bank Term Debt: Priced at Benchmark (Repo/MCLR) + 200–350 bps = **8.75% to 11.00% p.a.** all-in borrowing cost for top-tier multi-specialty hospitals and 4/5-star brand-flagged hotels.

Structured Private Credit AIF NCDs: Priced as All-In YTM = Cash Coupon (9.5%–12.0%) + PIK Accrual (2.5%–3.5%) + Exit Fee = **12.50% to 16.50% p.a. All-In YTM** for growth-stage hospital chains, boutique resorts, and promoter buyouts.

7.1 Suitable Industries & Business Profiles

Target Profiles: Hospitality & Healthcare Funding is best suited for:

Key Sub-Sectors: 1. Multi-Specialty & Single-Specialty Hospital Chains (Oncology, Cardiology, Eyecare, IVF)
2. Diagnostic & Pathology Laboratory Networks
3. Luxury, Upper-Upscale & Business Hotel Properties
4. Boutique Eco-Resorts & Heritage Tourism Properties
5. Senior Living, Assisted Care & Medical Rehabilitation Centers

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

8.1 Key Advantages and Limitations Matrix

The table below balances the structural benefits against the operational limitations of Hospitality & Healthcare Funding.

Dimension	Key Advantages & Strategic Value	Limitations & Operational Risks
Tenure & Amortization	Extended 10-to-15 year tenures match long physical asset life and reduce annual debt service.	Requires long-term capital commitment and strict compliance throughout tenure.
Tax Efficiency	100% capital expenditure tax deduction under Section 35AD boosts early-year cash retention.	Section 35AD requires minimum 100 beds (hospitals) or 2-star+ rating (hotels).
Equity Protection	Zero equity dilution for founders compared to bringing in private equity partners.	Higher fixed debt service drag during economic downturns or low-occupancy seasons.
Asset Creation	Creates high-value commercial real estate, specialized infrastructure & diagnostic tech.	High initial CapEx and vulnerability to technology obsolescence (medical gear).
Cash Flow Isolation	TRA Escrow structure ring-fences room/patient revenues and automated debt servicing.	Strict cash waterfall limits promoter cash withdrawals until DSRA is fully funded.

Strategic Balance: Understanding these trade-offs allows developers and promoters to select the optimal capital mix between senior bank debt, private credit, and internal equity.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 9: Comprehensive Real-World Case Study (INR 300 Crore Blended Facility)

9.1 Borrower & Project Context

Borrower Profile: Vanguard Medicare & Hospitality Private Limited (VMHPL) is a prominent healthcare and hospitality enterprise in South India, operating a 250-bed Tertiary Care Hospital and a co-located 120-key Business Hotel & Wellness Resort in Hyderabad. Pre-funding financial standings were as follows:

Baseline Standings: 1. Hospital Annual Revenue: INR 140.00 Crores (ARPOB: INR 38,000/day; Occupancy: 64%)
2. Hotel Annual Revenue: INR 40.00 Crores (ADR: INR 6,500/night; RevPAR: INR 4,200; Occupancy: 65%)
3. Combined Consolidated Revenue: INR 180.00 Crores | Combined EBITDA: INR 45.00 Crores (25.0% Margin)
4. Existing Debt: INR 50.00 Crores high-cost short-term debt @ 13.50% p.a.

9.2 Project Expansion Plan & Funding Structure

Expansion Scope: To capture growing regional demand for advanced cancer care and medical tourism, VMHPL initiated an INR 300.00 Crore expansion plan: (a) Constructing a new 150-bed Advanced Oncology & Robotic Surgery Block (INR 160 Cr), (b) Acquiring a Linear Accelerator & Surgical Robot (INR 60 Cr), (c) Refinancing INR 50 Cr existing high-cost debt, and (d) Working capital and resort refurbishment (INR 30 Cr).

Capital Stack Architecture: VMHPL secured a blended **INR 300.00 Crore Debt Facility** across two structured tranches:

Tranche A - Senior Bank Infrastructure Debt: INR 200.00 Crores (66.7% of debt) @ 9.50% p.a., 12-Year Amortization with a 24-Month Principal Moratorium during construction, secured by a 1st legal mortgage on real estate.

Tranche B - Structured Private Credit NCDs (Category II AIF): INR 100.00 Crores (33.3% of debt) @ 10.50% Cash Coupon + 3.50% PIK + 1.50% Exit Fee = 15.50% All-In YTM (7-Year Tenure, 24-Month Moratorium), secured by medical equipment hypothecation, TRA escrow, and 2.50x asset cover.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

9.3 5-Year Financial Scaling & Cash Flow Waterfall Model

The table below models VMHPL's operational scaling, revenue expansion, EBITDA growth, debt service coverage, and enterprise valuation over a 5-year post-expansion timeline.

Financial Metric (INR Cr)	Year 1 (Const.)	Year 2 (Ramp-up)	Year 3 (Stable)	Year 4 (Peak)	Year 5 (Maturity)
Total Operational Hospital Beds	250 Beds	320 Beds	400 Beds	400 Beds	400 Beds
Hospital Occupancy % / ARPOB (INR)	65% / ₹40,000	70% / ₹45,000	74% / ₹49,000	76% / ₹52,000	78% / ₹54,000
Hotel Occupancy % / RevPAR (INR)	66% / ₹4,500	70% / ₹5,200	72% / ₹5,800	75% / ₹6,400	76% / ₹6,800
Consolidated Revenue (INR Cr)	₹210.00 Cr	₹310.00 Cr	₹430.00 Cr	₹510.00 Cr	₹580.00 Cr
Consolidated EBITDA (INR Cr)	₹52.50 Cr	₹80.60 Cr	₹116.10 Cr	₹137.70 Cr	₹156.60 Cr
Senior Debt Service (Tranche A)	₹19.00 Cr (Int)	₹19.00 Cr (Int)	₹31.00 Cr (P+I)	₹29.50 Cr (P+I)	₹28.00 Cr (P+I)
Private Credit Service (Tranche B)	₹10.50 Cr (Int)	₹10.50 Cr (Int)	₹22.50 Cr (P+I)	₹21.00 Cr (P+I)	₹19.50 Cr (P+I)
Debt Service Coverage Ratio (DSCR)	1.78x	2.73x	2.17x	2.73x	3.30x
Enterprise Valuation @ 10x EBITDA	₹525.00 Cr	₹806.00 Cr	₹1,161.00 Cr	₹1,377.00 Cr	₹1,566.00 Cr

Case Study Conclusion: By Year 5, VMHPL expanded consolidated revenue to INR 580.00 Crores and EBITDA to INR 156.60 Crores, strengthening DSCR to 3.30x. The company created an enterprise valuation of INR 1,566.00 Crores while preserving 100% promoter equity control.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 10: Legal Enforcement, Default Protocols & IBC 2016 Integration

10.1 SARFAESI Act 2002 Enforcement Mechanics

SARFAESI Enforcement: Upon an uncured Payment Default (typically 90 days overdue resulting in NPA classification), lenders trigger legal enforcement under the SARFAESI Act, 2002:

1. Section 13(2) Demand Notice: Issuance of a 60-day statutory demand notice under Section 13(2) demanding full debt discharge.

2. Section 13(4) Possession Powers: If unfulfilled after 60 days, lenders exercise powers under Section 13(4) to take physical possession of commercial real estate land, hospital buildings, and hotel properties, appointing receiver managers.

10.2 Insolvency and Bankruptcy Code (IBC) 2016 & Service Continuity

IBC Statutory Status: Lenders holding debentures or term debt qualify as Financial Creditors under Section 5(8) of the IBC 2016, with statutory rights to initiate Corporate Insolvency Resolution Process (CIRP) before the NCLT under Section 7 for defaults exceeding INR 1.00 Crore.

Healthcare Service Continuity: Crucially, for healthcare assets, public health regulations and IBC guidelines mandate the statutory continuity of essential medical services during insolvency proceedings. Resolution Professionals (RPs) must maintain hospital operations without disrupting in-patient ICUs, emergency care, or ongoing surgeries.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Chapter 11: Rookie Mastery Examination (15 High-Yield Test Q&As)

This section contains 15 structured, highly detailed practice questions and comprehensive answers designed to verify full conceptual and practical mastery of Hospitality & Healthcare Funding.

Q1: What defines Hospitality & Healthcare Funding and how does infrastructure status impact loan terms?

Hospitality & Healthcare Funding represents specialized wholesale credit and structured term debt for hospitals, diagnostic chains, hotels, and resorts. Infrastructure status granted to hospitals with 100+ beds and hotels rated 3-star and above enables commercial banks to extend longer loan tenures (up to 15–20 years), assign lower Risk-Weighted Assets (RWA), and provide 24-to-36 month principal moratoriums matching construction and ramp-up phases.

Q2: What is Section 35AD of the Income Tax Act 1961 and how does it benefit healthcare and hospitality borrowers?

Section 35AD provides a 100% tax deduction on all capital expenditure incurred (excluding land, goodwill, and financial instruments) during the financial year for building and operating new hospitals with at least 100 beds or new 2-star or higher category hotels. This significantly reduces taxable profits in early operational years, boosting cash retention for debt service.

Q3: Explain the core operational metrics used in healthcare credit underwriting: ARPOB and ALOS.

ARPOB (Average Revenue Per Occupied Bed) measures daily inpatient revenue efficiency, calculated as Total Inpatient Revenue divided by Occupied Bed Days. Target benchmarks range from INR 25,000 to INR 65,000+/day. ALOS (Average Length of Stay) measures patient turnover efficiency, calculated as Total Inpatient Days divided by Total Admissions. An optimal ALOS of 3.5 to 5.0 days ensures high bed turnover without compromising clinical care.

Q4: Explain the core operational metrics used in hospitality credit underwriting: ADR and RevPAR.

ADR (Average Daily Rate) measures the average rental revenue earned per occupied room per day. RevPAR (Revenue Per Available Room) measures total room revenue yield relative to total available rooms, calculated as ADR multiplied by Occupancy Rate % (or Total Room Revenue

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

divided by Total Available Rooms). Healthy benchmark RevPARs range from INR 3,200 to INR 18,000+/night.

Q5: How does a Trust & Retention Account (TRA) Escrow cash waterfall protect lenders in healthcare and hospitality debt?

A TRA Escrow agreement mandates that 100% of cash collections (patient billings, TPA/insurance claims, room card swipes, banquet deposits) flow into a lender-controlled escrow account. Cash is disbursed according to a strict legal waterfall: (1) Statutory Taxes, (2) Pre-approved O&M, (3) Senior Debt Service, (4) Subordinated Debt Service, (5) DSRA replenishment, (6) CapEx reserves, (7) Cash sweeps (if triggered), and (8) Surplus release to promoters.

Q6: What statutory clearances are mandatory for operating radiology and oncology departments in a hospital?

Operations of Radiology, CT, PET-CT, and Radiation Oncology facilities require mandatory radiation safety clearances and equipment registration from the Atomic Energy Regulatory Board (AERB) under the Atomic Energy Act, alongside compliance with Bio-Medical Waste Management Rules 2016.

Q7: What statutory licenses are essential for commercial hotel operations in India?

Commercial hotels require FSSAI Food Safety Licenses for kitchens, Excise/Bar Licenses for alcohol service, Local Municipal Building & Commercial Occupation Certificates (OC), Fire & Life Safety NOCs, and Coastal Regulation Zone (CRZ) clearances for beach resorts.

Q8: How does TPA and Health Insurance Receivables Discounting work for hospitals?

Hospitals often experience 45-to-90 day payment delays from Third Party Administrators (TPAs) and health insurers (CGHS, ECHS, PM-JAY, private insurers). Receivables discounting provides a revolving credit line where lenders advance 80%–85% upfront cash against verified insurance claims, with TPA remittances routed directly into a escrow account to extinguish debt.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Q9: What is the typical asset cover ratio (ACR) required for institutional hospital and hotel loans?

Lenders require a minimum Asset Cover Ratio (ACR) of 2.00x to 2.50x, meaning the net realizable value of mortgaged real estate land, commercial buildings, and hypothecated medical/hotel equipment must equal at least 200% to 250% of the sanctioned loan facility.

Q10: Explain the difference between Commercial Bank Term Debt and Structured Private Credit AIF NCDs in this sector.

Commercial Bank Term Debt offers lower interest rates (8.75%–11.00% p.a.) and long tenures (10–15 years) but enforces strict collateral and covenant rules. Structured Private Credit AIF NCDs carry higher yields (12.50%–16.50% All-In YTM) using split coupons (Cash + PIK + Exit Fee), offering flexible moratoriums and higher leverage for growth, brownfield expansions, or promoter buyouts.

Q11: What is a Debt Service Reserve Account (DSRA) and how is it structured?

A DSRA is a ring-fenced liquidity buffer held in liquid bank fixed deposits pledged to the lender. It maintains 3 to 6 months of future principal and interest payouts. If operational cash flow drops temporarily, the lender can draw from the DSRA to prevent an NPA default.

Q12: What statutory perfection requirements apply to security charges under Companies Act 2013?

Under Section 77 of the Companies Act 2013, all security charges created over corporate assets (mortgages, hypothecations, escrow charges) must be registered with the Registrar of Companies (ROC) on Form CHG-1 within 30 days of creation, uploaded to CERSAI, and registered on NeSL.

Q13: How does the IBC 2016 treat healthcare assets during insolvency proceedings?

Under IBC 2016, term debt/debenture holders hold Financial Creditor status with Section 7 CIRP initiation rights before NCLT. Crucially, public health regulations and IBC guidelines mandate that Resolution Professionals maintain statutory continuity of essential medical services without disrupting patient ICUs or emergency care during restructuring.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.

Q14: What is Lease Rental Discounting (LRD) in the context of operating hotel assets?

LRD allows hotel owners with stable occupancy (>65%) to securitize long-term lease agreements (or predictable room credit card receipts). The lender advances lump-sum debt calculated as the present value of future lease/room cash flows discounted at a corporate interest rate, with tenant/room receipts deposited directly into an escrow account.

Q15: What are the primary risk factors underwriters evaluate in greenfield hotel developments?

Underwriters evaluate construction cost overruns, delay risks in securing municipal/environmental NOCs, regional room oversupply risk, catchment area ADR growth, and the credibility of the appointed Hotel Management Agreement (HMA) brand operator.

Copyright Notice: © 2026 www.loanbazaronline.com. All rights reserved.